

DISTINCT CHARACTER

DISTINCT CHARACTER RESOURCE LIBRARY | FOUNDATION

Body Signal Index

FOUNDATION REFERENCE

Somatic literacy reference for reading body signals with precision

Asset ID: DC-R04-BSI | Revision v0.1

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Body Signal Index is an educational and self-development resource within the Distinct Character ecosystem. It is designed for somatic literacy, behavioral governance, and structured self-observation.

This resource does not provide medical advice, diagnosis, psychotherapy, psychiatric care, crisis intervention, legal advice, or financial advice. It is not a substitute for individualized care from a licensed medical, mental health, or other qualified professional.

If you are experiencing acute distress, medical symptoms, self-harm thoughts, dissociation, panic, severe depression, eating-disorder symptoms, or any condition requiring clinical support, stop using the resource and contact an appropriate licensed professional or emergency service in your location.

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BODY SIGNAL INDEX

Distinct Character | Biological Infrastructure Companion

A somatic literacy reference for reading the body with precision. Not a assessment-oriented instrument. Use in conjunction with the Somatic Architecture Cycle.

Signal Reference Table

SIGNAL	POSSIBLE FRAMEWORK MEANING	FIRST RESPONSE
TENSION AND BREATH		
Jaw tension or clenching	Suppressed speech, held boundary, or chronic low-level stress.	<i>Jaw release; name what is being held back.</i>
Shallow or held breath	Threat response active; window of tolerance narrowing.	<i>Elongated exhale; orienting breath.</i>
Chest tightness	Anxiety, unprocessed emotion, or grief.	<i>Breath work; name the emotion precisely.</i>
Neck or shoulder holding	Responsibility load, over-efforting, or performance pressure.	<i>Physical release; assess overfunctioning pattern.</i>
ENERGY AND CAPACITY		
Heavy limbs or fatigue	Depletion, recovery deficit, or parasympathetic shutdown.	<i>Rest without guilt; assess sleep and load.</i>
Flat affect or heaviness	Approaching or inside depletion; possible depression signal.	<i>Rest; medical check-in if persistent.</i>
Irritability without clear cause	Elevated cortisol load, depleted capacity, or unmet need.	<i>Do not make decisions; regulate first.</i>
ACTIVATION AND DYSREGULATION		
Restlessness	Sympathetic activation, unmet need, or suppressed movement.	<i>Movement discharge; then stillness.</i>
Buzzing or hypervigilance	Sympathetic overdrive; threat perception elevated.	<i>Sensory reduction; slow movement; stillness.</i>
SHUTDOWN AND DISSOCIATION		

SIGNAL	POSSIBLE FRAMEWORK MEANING	FIRST RESPONSE
Numbness or disconnection	Dissociation, overwhelm past threshold, or freeze response.	<i>Grounding; gentle orienting; safe co-regulation.</i>
SOMATIC AND DIGESTIVE		
Digestive disruption	Elevated cortisol, nervous system activation, or suppressed stress.	<i>Regulate before eating; assess allostatic load.</i>

Usage Protocol: The Somatic Architecture Cycle

When a signal is identified, move through these steps in sequence. The cycle does not require a single sitting. Notice and Name happen in seconds. Adjust may take days.

- 01** NOTICE: Identify the signal before labeling it. Precision precedes interpretation.
- 02** NAME: Match the signal to this index. Use the framework language, not casual shorthand.
- 03** REGULATE: Apply the indicated first response. Do not skip to analysis while the body is still activated.
- 04** RECOVER: Allow full restoration before re-engaging with high-demand work or decisions.
- 05** ADJUST: Use the signal as data. Make a structural change to load, environment, or schedule if the signal repeats.

Signal Interpretation Notes

Signals that appear in clusters indicate broader regulation load, not isolated symptoms. If three or more signals from the Activation or Tension categories present simultaneously, treat this as a possible Stage 2 or Stage 3 depletion indicator.

Recurring signals that return within 24 to 48 hours after regulation indicate a structural problem in load, environment, or recovery architecture, not a personal regulation failure. Refer to the Depletion and Recovery Map.

Flat affect, numbness, or persistent heaviness lasting more than three days should be assessed by a licensed professional. This index does not replace clinical support.

For those managing chronic illness: signals may not follow the standard intensity-to-cause pattern. Use this index as a starting reference and calibrate against your known baseline, not population norms.